

Is It Time?

There's a difference between a person who's declining and someone who can no longer safely live alone — and months can be lost in the fog between.

This isn't a test to pass or fail. It's a way to turn a vague impression into something you can see — and put in front of someone who may not be ready to see it yet.

Answer honestly, and score each question as an actual number. That's what turns the feeling you've been carrying around into something you can act on.



BEFORE YOU BEGIN

If your loved one is having a medical emergency right now, call 911 or go to the ER. Come back to this assessment when things are stable.

If there's someone else who knows your loved one well — a sibling, a spouse, a friend — it's worth asking them to score this separately too, before comparing notes. Where you agree helps validate what you're seeing. If they score it more favorably than you, that gap may mean you're minimizing, or it may mean something worth paying closer attention to. If they score it more severely than you, listen. They may have been waiting for you to ask. If there's no one else, that's fine, this works just as well on its own.

One thing before you start: there's no single score at the end. Some gaps can be weighed against everything that's going right; some can't. So instead of a total, you'll get a plain answer: what to do next.

HOW TO SCORE

The same three-point scale applies to all ten questions.

0

Not a concern

This is not happening, or happens very rarely

1

Some concern

This has started, comes and goes, or you've noticed it but aren't sure yet

2

Significant concern

This is happening consistently, or has happened in a way that scared you

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THE TEN QUESTIONS

Safety

- 1** Has any of this happened in the past few months? Check any that apply, then score the question as a whole.

0

1

2

 - ☐ A fall or near-fall
 - ☐ The stove, oven, or faucet(s) left on
 - ☐ An injury or bruise they couldn't explain
 - ☐ Wandered, got lost, or left the house alone unexpectedly
 - ☐ A neighbor or friend had to step in or reach out about something safety-related
- 2** If something went wrong right now, a fall or a medical emergency, could your loved one recognize it and get help?

0

1

2
- 3** In the past year, has your loved one been to the ER or been admitted to the hospital? Score based on how many times: none, once, or more than once.

0

1

2
- 4** Is your loved one still driving? If so, have there been near-misses or accidents, getting lost on familiar routes, or people quietly avoiding riding along?

0

1

2

Cognition

- 5** Can your loved one accurately tell you: what medications they take, how often, and why?

0

1

2
- 6** If a stranger called claiming to be a grandchild in trouble, or a bank asking to verify account details, would your loved one recognize it as a scam and hang up?

0

1

2
- 7** Have you noticed repeated stories, forgotten recent conversations, or confusion about the day, date, or where they are?

0

1

2

THE TEN QUESTIONS

Self Care

8 Are meals, medications, and bills being handled consistently? Does anything in the mail, the fridge, or the calendar suggest otherwise? 0 1 2

9 Has personal care changed in any of these ways? Check any that apply, then score the question as a whole. 0 1 2

- ☐ Bathing or dressing has become difficult
- ☐ Using the bathroom independently has become difficult
- ☐ Getting in or out of bed or a chair safely has become difficult
- ☐ Eating enough has become a struggle

Caregiver Sustainability

10 Is keeping things running as they are wearing you, or another caregiver in this person's life, down in a way that can't continue indefinitely? 0 1 2

FINDING YOUR ANSWER

The four domains aren't a sum you total up. They're a judgment about which gaps you can safely live with, and which you can't. Work down these steps in order and stop at the first one that applies to you.

- | | | |
|---------------|---|------------------------|
| STEP 1 | Did you mark a 2 on Question 2 or Question 5? | Yes → Outcome 1 |
| STEP 2 | Did you mark a 2 on two or more of the Safety questions? | Yes → Outcome 1 |
| STEP 3 | Do two or more of Safety, Cognition, or Self Care have a 2? | Yes → Outcome 2 |
| STEP 4 | Is anything else marked at all, a 1 or a 2, anywhere? | Yes → Outcome 3 |
| STEP 5 | Otherwise. | Outcome 4 |

YOUR OUTCOME

A NOTE BEFORE YOU READ ON

Distance, privacy rules, or a strained relationship can all get in the way of this. If you can't reach the doctor directly — you're not an authorized contact, or your loved one won't go — that's common, and it's exactly what the Starting the Conversation guide helps with.

OUTCOME 1

Talk to the doctor this week.

If this came on suddenly, in the last few days, or your loved one seems unwell right now, don't wait for an appointment: call today, or use urgent care or the ER if you can't reach the doctor's office. A sudden change is often something treatable, but it needs to be evaluated quickly, not watched.

If this has been building over weeks or months, call this week, and ask specifically to address what's concerning you: a fall-risk assessment if a safety question set this off, a full medication review if it was the medications question, or an evaluation of memory and judgment if it was a pattern of safety concerns. Naming what you're worried about gets a more useful visit than a general checkup. This is a medical question before it's a housing question.

OUTCOME 2

Time to start the real work.

Not necessarily time to move tomorrow, but time to stop watching and start planning. First: mention this pattern to your loved one's doctor, at the next visit or sooner if you can get one. Some of what looks like decline is treatable and worth ruling out before you assume it's permanent. Second: start the personal side. Download the free Starting the Conversation guide and use it to bring your loved one into this, rather than deciding on their behalf. (*The Long Road* walks through how to gauge whether you have three weeks or six months, if you want a fuller picture.)

OUTCOME 3

Keep watching.

Something specific stood out, even if nothing else did. This outcome can trigger three ways, and they need different next steps. Read the one that fits what you marked.

If the trigger was Safety, Cognition, or Self Care

Go back and find what concerned you, whether it's one clear answer or a few smaller ones spread across different areas. Write down exactly what you saw, the date, and the specific example, not just "they seemed off." If you attend, mention it at their next routine doctor's visit. A primary care visit is a low-key way to get a second set of eyes on something you're not sure about yet. Put a date on your calendar one month out and retake this assessment then. If the same thing shows up again, or spreads further, that's your answer.

If Caregiver Sustainability is marked

This one isn't about your loved one, it's about you, and "watch and reassess in a month" isn't a real answer to it. What you need isn't more tracking, it's less load: a break, someone else taking a shift, a conversation with your own doctor about how this is affecting you, or simply saying out loud to another person that you can't keep carrying this alone. Revisit this question specifically whenever your own capacity changes, not on a fixed schedule.

If both are marked

Follow both steps above. Multiple things can be true at once, and often are: something about your loved one is worth watching, and the load on you is also real. You don't have to choose between them.

OUTCOME 4

Not yet, and that's real information.

NEXT STEP

Nothing here crossed into real concern. That's a trustworthy answer, not a guess, and not a reason to keep checking on a schedule. Don't put this on your calendar. Their next annual wellness visit is a natural place to mention what you've been noticing, even at this level, a good primary care relationship catches more over time than periodic panic does. Otherwise, come back to this only if something actually changes: a fall, a hospitalization, a diagnosis, a bad week that doesn't end.



ABOUT THE AUTHOR

The Geriatrician's Guide is founded and edited by **Michael D. Genualdi, MD**, a board-certified geriatrician and internist. After completing his residency and fellowship training at Stanford, he spent years caring for older adults across the full range of care settings — including serving as medical director of a primary care practice embedded in dozens of senior living communities. He built this resource to share what that experience has taught him, so people can care for a loved one with confidence instead of guesswork. The Geriatrician's Guide does not accept compensation of any kind from senior living operators, placement services, or referral networks.

WHERE TO GO NEXT

FREE GUIDE

Starting the Conversation

A step-by-step guide to raising this with your loved one — so the decision is made with them, not around them.

Get the guide →

THE COMPLETE RESOURCE

The Long Road

A Geriatrician's Complete Guide to Senior Care Decisions

Chapter 4 goes deeper into the ten questions you just answered — how to see what's actually happening, how much time you have before this becomes urgent, and what to do if you're met with resistance. When you're ready, the rest of the book is waiting: the financial mechanics, what to look for once you're touring, and how to keep advocating for your loved one through whatever comes next.

Get The Long Road →



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No placement commissions.
No facility money. Ever.